



Patient Detail:

Huma .

Age/Sex

Mobile #

42 (Y) / F - 03118942364

Registration Location:

Shaheed-e-Millat Road, Karachi

Registration Date:

23-Oct-2025 17:39

Reference:

Saylani Medical & Diagnostic

Consultant:

Self

Patient Number:

798001-25-2036765532

Case Number:

177027-23-10



Note : , , ,

Department of Radiology

Reporting Date/Time: 25-Oct-2025 11:20

CT Chest, Abdomen and Pelvis with contrast

Clinical History:

Left breast lump, biopsy proven infiltrating ductal carcinoma

Findings:

CT ABDOMEN:

Contrast enhanced CT shows smooth surfaced liver without any mass.

No intra or extra hepatic biliary duct dilatation.

No radio-opaque biliary calculus.

No mass in gall bladder, pancreas, spleen and adrenal glands.

Both kidneys, ureters and urinary bladder are also normal. Small right renal cortical cyst observed

No hiatal hernia. No gastroduodenal mass seen.

No abnormal dilatation or wall thickening of the bowel loops.

Mesenteric and hepatic vessels are patent.

Subcentimeter sized mesenteric lymph nodes observed predominantly along right ileocolic chain
index one measures 11 mm in short axis dimension

No ascites.

Uterus and both adnexa appear unremarkable

No ventral abdominal wall hernia.

Visualized skeleton is negative for any deposits or bone destruction. Bone scan suggested

CT CHEST:

A lobulated mildly enhancing 2.3 X 2.2 cm lesion identified in medial quadrant of left breast not infiltrating the underlying chest wall or overlying skin. Enlarged level I left axillary lymph node observed measuring 2.5 x 1.3 cm.

No discrete lesion seen in right breast. No evidence of right axillary lymph node enlargement.

No definite pulmonary nodule or mass seen in both lungs to suggest pulmonary metastasis

No consolidation, cavitation, bronchiectasis or honeycombing.

No mediastinal lymph node enlargement.

No pneumothorax, emphysema or pericardial effusion. No bilateral pleural effusion.

Trachea, main bronchi and thoracic esophagus are normal.

Heart and great vessels are also normal.

IMPRESSION

A lobulated mildly enhancing lesion in medial quadrant of left breast not infiltrating the underlying chest wall or overlying skin associated with enlarged level I left axillary lymph node, findings are compatible with biopsy-proven infiltrating ductal carcinoma with left axillary metastatic node.

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Consultant Interventional Radiologist

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Diagnostic Imaging Specialist

Dr. Omar Chughtai
MBBS, MD, FCAP
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and Clinical Pathology

Dr. A. S. Chughtai
MBBS, MIAC, M.Phil.,
FCPS, FRC Path Consultant Pathologist



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Note : , , ,

No definite metastatic pulmonary, hepatic or adrenal deposit observed

SUGGESTION

This interpretation is based on presenting clinical context and available information.

Correlation with the clinical assessment, previous imaging and laboratory investigations is suggested. The study can be reviewed in case of any clinical query.

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